

STANDARD OPERATING PROCEDURE

Document Title: SOP on Health Insurance Claim Adjudication

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Applicable To: myRelief Basic, Standard, and Premium plans

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Status: Fictional document — AI Engineering Portfolio Project

1. PURPOSE

This SOP defines the standard procedure for the intake, verification, adjudication, and finalization of health insurance claims submitted under myRelief Health Insurance products, whether via Cashless or Reimbursement mode. It exists to ensure that claim decisions are accurate, consistent, timely, auditable, and compliant with applicable regulatory requirements, regardless of which system or personnel handles a given claim.

2. SCOPE

This SOP applies to all claims — Medical, Surgical, Day Care, and Follow-up Care — submitted under any myRelief health insurance plan (Basic, Standard, Premium), through Network Hospitals (Cashless) or any eligible Hospital (Reimbursement). It covers the process from claim intake through final decision, including verification, escalation, and audit review. It does not cover policy issuance, underwriting, or premium collection, which are governed by separate SOPs.

3. DEFINITIONS

Term	Definition
Claim	A formal request submitted for payment of eligible medical expenses under a myRelief policy
Cashless Facility	A claim mode where payment is made directly to the Network Hospital, subject to pre-authorization
Reimbursement	A claim mode where the Insured Person pays first and is reimbursed upon claim approval
Pre-authorization	Prior approval sought from myRelief before or shortly after Cashless hospitalization
Enhancement Request	A request to extend previously approved coverage days/amount during ongoing hospitalization
Primary Adjudication Function	The function responsible for the initial review and decision recommendation on a claim, based on policy terms, submitted evidence, and applicable regulation
Secondary Verification Function	An independent review function that re-examines a claim routed to it, verifying clause applicability, calculations, and reasoning before a decision is finalized
Risk-Routing Gate	The checkpoint at which a claim is classified as Fast-Path or Slow-Path, based on defined objective criteria

Term	Definition
Human Adjudicator	A qualified claims officer authorized to make or override a final decision on escalated claims
Exception/Hardship Signal	Any claimant-provided context suggesting extenuating circumstances not directly addressed by standard policy wording

4. ROLES AND RESPONSIBILITIES

Role	Responsibility
Claims Intake Function	Receives claim submissions; performs document completeness checks
Primary Adjudication Function	Cross-checks claim against policy terms, regulatory rules, and applicable sub-limits; produces an initial decision recommendation with justification
Secondary Verification Function	Independently re-verifies Slow-Path claims before finalization
Human Adjudicator	Reviews escalated claims; makes or overrides final decisions where the Secondary Verification Function does not concur, or where an Exception/Hardship Signal is present
Quality & Compliance Function	Conducts mandatory random audit sampling of Fast-Path decisions; monitors decision quality and regulatory compliance on an ongoing basis
Claims Operations Management	Owns this SOP; reviews and revises Risk-Routing Gate criteria and thresholds based on audit and appeal outcomes

Note: The Primary Adjudication Function and Secondary Verification Function may be fulfilled by automated systems, human staff, or a combination of both, at myRelief's operational discretion. This SOP defines the procedure and required outcomes at each step, independent of the specific system or tooling used to fulfill each function.

5. PROCEDURE

5.1 Claim Intake

5.1.1 Cashless — Pre-authorization

- Medical (non-surgical): raised on day of admission; backdating permitted up to 5 days (public/trust hospitals) or 3 days (private hospitals) if missed; not permitted on date of discharge.
- Surgical (elective): raised prior to surgery. Where advance pre-authorization is not possible due to a technical issue, the Network Hospital must intimate myRelief within 3 days (private) or 5 days (public/trust); failure to do so results in the claim being provisionally flagged for manual review.
- Surgical (emergency): may be raised after surgery, but not on date of discharge.
- Day Care: raised on the same day as the procedure.
- Follow-up Care: raised with reference to the original hospitalization episode's discharge summary.

- First day of an approved admission is auto-approved upon valid submission. Extensions require an Enhancement Request, raised no earlier than 24 hours after initial approval, with updated clinical notes.
- Multiple procedures performed in a single Operation Theatre session are reimbursed at 100% (1st procedure), 50% (2nd procedure), and 25% (3rd and subsequent procedures) of applicable package rate.

5.1.2 Reimbursement

- Required documents: itemized bills, discharge summary, investigation/diagnostic reports, and prescriptions as applicable to the claim type.
- Submission deadline: within 30 days of date of discharge.

5.1.3 Service Standards (Cashless)

- Decision on a cashless authorization request: within 1 hour of receipt from the Network Hospital.
- Final discharge authorization: within 3 hours of receipt of final discharge bill.

5.2 Verification (Primary Adjudication Function)

5.2.1 Document Completeness Check — Confirm all required documents (per 5.1.1/5.1.2) are present and adequate. Where a document is missing, the claim is placed on Document Deficiency Hold; claimant/hospital is notified with a 7-day window to supply the missing item.

5.2.2 Policy & Coverage Cross-Check — Verify: policy active status, applicable waiting periods (initial, Pre-existing Disease, specific-disease list) per the Policy Document, applicable exclusions, Room Rent capping, Co-payment (if applicable to plan), and remaining Sum Insured (including Restoration eligibility per plan).

5.2.3 Cross-Signal Verification — Compare claim details against claimant's prior claim history and Reasonable and Customary Charges benchmarks. A claim is flagged for further scrutiny where treatment cost significantly exceeds benchmark without clinical justification, where 3 or more unrelated claims occur within the same Policy Year, or where the hospital/claimant has a documented history of discrepancies.

5.2.4 Decision Recommendation — The Primary Adjudication Function produces one of: Approve, Partial Approve (with itemized justification for any deduction), Deny (citing the specific applicable clause), or a recommendation for escalation, along with a structured justification referencing the applicable Policy and regulatory clauses.

5.3 Risk-Routing Gate

Each claim is routed based on objective criteria evaluated during Section 5.2 — not on any self-assessment by the Primary Adjudication Function of its own recommendation:

Fast-Path — all of the following hold: documents complete and adequate; no waiting-period/exclusion ambiguity; no cross-signal flag (5.2.3); no Exception/Hardship Signal present; claimed amount below the defined low-risk threshold (Section 5.6).

Slow-Path — any of the following hold: document adequacy uncertain; cross-signal flag raised; Exception/Hardship Signal present; claimed amount at or above threshold; waiting-period/exclusion applicability ambiguous.

5.4 Fast-Path Finalization

The Primary Adjudication Function's recommendation is finalized and issued directly. No secondary verification or human review occurs in real time. Subject to mandatory random audit sampling (Section 5.6).

5.5 Slow-Path Finalization

- The Secondary Verification Function independently re-examines the claim: retrieved evidence, applicable clauses, and calculations, without reference to the Primary Adjudication Function's confidence in its own

recommendation.

- Where the Secondary Verification Function concurs, the decision is finalized and issued.
- Where the Secondary Verification Function does not concur, confidence remains low, or an Exception/Hardship Signal is present, the claim is escalated to a Human Adjudicator. The claim shall not be auto-denied in these cases; the Human Adjudicator is presented with the relevant policy clauses, claimant context, and a recommendation.

5.6 Quality Assurance — Audit Sampling and Threshold Review

- A minimum of 3–5% of all Fast-Path decisions, selected at random independent of any flag, shall be reviewed daily by the Quality & Compliance Function.
- This sampling exists to identify claims the Risk-Routing Gate may have misclassified, which the Gate cannot detect on its own.
- Outcomes from audit sampling and from Grievances/Appeals (Section 6) are aggregated by Claims Operations Management on an ongoing basis and used to review and, where necessary, revise the Risk-Routing Gate's threshold and flag criteria (Section 5.3).

5.7 Audit Trail

Every claim's complete adjudication path — documents reviewed, clauses referenced, calculations applied, routing decision, and reasoning for the final outcome — shall be logged and retained for audit and regulatory review.

6. GRIEVANCE AND APPEAL

A claimant disagreeing with a Deny or Partial Approve decision may request a review through myRelief's grievance process. Reviews shall be conducted by a Human Adjudicator not involved in the original decision.

7. REFERENCES

- myRelief Policy Documents — Basic, Standard, Premium (Document IDs: MYRELIEF-BASIC-001, MYRELIEF-STANDARD-001, MYRELIEF-PREMIUM-001)
- IRDAI Master Circular on Health Insurance Business (Ref: IRDAI/HLT/CIR/PRO/84/5/2024, dated 29 May 2024)
- Structural reference (pre-authorization, enhancement, multi-procedure payout): Megha Health Insurance Scheme, "SOP on Claim Adjudication" (publicly published)

8. REVISION HISTORY

Version	Date	Description of Change	Author
1.0	—	Initial draft — internal adjudication workflow	Claims Operations

Version	Date	Description of Change	Author
2.0	—	Restructured to standard SOP format (Document Control, Purpose, Scope, Definitions, Roles & Responsibilities, Procedure, References, Revision History); added Risk-Routing Gate, Secondary Verification Function, mandatory audit sampling, and feedback loop; removed all system/vendor-specific references in favor of role-based function descriptions	Claims Operations

9. APPROVAL

Role	Name	Signature	Date
Author	—	—	—
Reviewer	—	—	—
Quality & Compliance Approver	—	—	—
Management Approver	—	—	—

This document is a fictional internal Standard Operating Procedure created for an educational AI engineering portfolio project. Format follows standard SOP conventions (Document Control, Purpose, Scope, Definitions, Roles & Responsibilities, Procedure, References, Revision History, Approval). Pre-authorization, enhancement, and multi-procedure payout structure (Section 5.1.1) are adapted from the publicly published Megha Health Insurance Scheme "SOP on Claim Adjudication." Cashless service-level timelines (Section 5.1.3) reflect the real IRDAI Master Circular on Health Insurance Business, 2024. All other procedural detail, roles, and thresholds are fabricated by myRelief for this project and do not represent any real insurer's actual internal procedure. Role names (e.g., "Primary Adjudication Function," "Secondary Verification Function") are intentionally implementation-agnostic; the specific systems or models fulfilling these roles are documented separately in the project's technical architecture documentation, not in this SOP.